SACTCHECK · TREATMENT PASSPORT

FOLFIRI + bevacizumab

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Day 1: bevacizumab, irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

Local · MON–FRI 8AM–5PM

Insert numbers here

Acute Oncology Service

Local · WEEKENDS / BANK HOLIDAYS 9AM–5PM

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00449/

Full patient information, sources and visual timeline. No identifiers are encoded.

Draft for clinical, pharmacy and patient/PPI review

NCCP 00449 V6A · ONE CYCLE, ONE PAGE

FOLFIRI + bevacizumab

Cycle number: _____ Start date: _____ Next review: _____

Bevacizumab · Anti-VEGF targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999
Headache	☐ None	☐ Mild and short-lived: record	☐ New / persistent headache: call	☐ Severe headache / visual change: urgent help	☐ Sudden worst headache / weakness: 112/999

Irinotecan · Topoisomerase I chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Folinic acid · Chemotherapy modulator

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
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Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999

Fluorouracil · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Hand-foot soreness	☐ None	☐ Redness or tingling without pain	☐ Pain limits usual tasks: call	☐ Pain limits washing / dressing: urgent review	☐ Acute severe illness: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

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